

LCD Reference Article

Billing and Coding Article

Billing and Coding: Polysomnography and Other Sleep Studies

A56903

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Contractor Information

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Wisconsin Physicians Service Insurance Corporation	MAC - Part A	05101 - MAC A	J - 05	Iowa
Wisconsin Physicians Service Insurance Corporation	MAC - Part B	05102 - MAC B	J - 05	Iowa
Wisconsin Physicians Service Insurance Corporation	MAC - Part A	05201 - MAC A	J - 05	Kansas

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Wisconsin Physicians Service Insurance Corporation	MAC - Part B	05202 - MAC B	J - 05	Kansas
Wisconsin Physicians Service Insurance Corporation	MAC - Part A	05301 - MAC A	J - 05	Missouri - Entire State
Wisconsin Physicians Service Insurance Corporation	MAC - Part B	05302 - MAC B	J - 05	Missouri - Entire State
Wisconsin Physicians Service Insurance Corporation	MAC - Part A	05401 - MAC A	J - 05	Nebraska
Wisconsin Physicians Service Insurance Corporation	MAC - Part B	05402 - MAC B	J - 05	Nebraska
Wisconsin Physicians Service Insurance Corporation	MAC - Part A	05901 - MAC A	J - 05	Alabama Alaska Arizona

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
				Arkansas
				California - Entire State
				Colorado
				Connecticut
				Delaware
				Florida
				Georgia
				Hawaii
				Idaho
				Illinois
				Indiana
				Iowa
				Kansas
				Kentucky
				Louisiana
				Maine
				Maryland
				Massachusetts
				Michigan
				Mississippi
				Missouri - Entire State
				Montana
				Nebraska
				Nevada

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
				New Hampshire
				New Jersey
				New Mexico
				North Carolina
				North Dakota
				Ohio
				Oklahoma
				Oregon
				Pennsylvania
				Rhode Island
				South Carolina
				South Dakota
				Tennessee
				Texas
				Utah
				Vermont
				Virginia
				Washington
				West Virginia
				Wisconsin
				Wyoming
Wisconsin Physicians Service Insurance Corporation	MAC - Part A	08101 - MAC A	J - 08	Indiana

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
Wisconsin Physicians Service Insurance Corporation	MAC - Part B	08102 - MAC B	J - 08	Indiana
Wisconsin Physicians Service Insurance Corporation	MAC - Part A	08201 - MAC A	J - 08	Michigan
Wisconsin Physicians Service Insurance Corporation	MAC - Part B	08202 - MAC B	J - 08	Michigan

Article Information

General Information

Article ID

A56903

Article Title

Billing and Coding: Polysomnography and Other Sleep Studies

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Article Type

Billing and Coding

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N/A

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CMS National Coverage Policy

N/A

Article Guidance

Article Text

The billing and coding information in this article is dependent on the coverage indications, limitations and/or medical necessity, and documentation requirements described in the associated LCD L36839 Polysomnography and Other Sleep Studies.

- Diagnostic testing will be considered for coverage if the patient has the symptoms or complaints of 1 of the conditions listed in L36839.
- Patients who undergo the diagnostic testing are not considered inpatients, although they may come to the facility in the evening for testing and then leave after testing is over. The overnight stay is considered an integral part of these tests. Documentation must support indications if the patient is admitted.
- Ordinarily, a single polysomnogram and electroencephalogram (EEG) can diagnose sleep apnea. If more than 1 such testing session is claimed, the carrier will require persuasive medical evidence justifying the medical necessity for the additional test (*CMS Publication 100-02, Medicare Benefit Policy Manual, Chapter 15, Section 70*).
- Ordinarily, a diagnosis of narcolepsy can be confirmed by 3 sleep naps. If more than 3 sleep naps are claimed; the A/B MAC (B) will require persuasive medical evidence justifying the medical necessity for the additional test(s). (*CMS Publication 100-02, Medicare Benefit Policy Manual, Chapter 15, Section 70*.)
- More than 1 HST per year interval would not be expected. If more than 1 HST session is performed for suspected OSA, persuasive medical evidence justifying the medical necessity for the additional tests will be required. Similarly, more than 2 PSG per year interval would not be expected. If more than 2 PSG sessions are performed for the diagnosis or adjustment of treatment of sleep, pervasive medical evidence justifying the medical necessity for the additional tests will be required upon request. When services are performed in excess of established parameters, they may be subject to review for medical necessity.
- The routine use of more than 1 PSG to titrate CPAP therapy would not be considered reasonable and necessary. If more than 1 CPAP titration PSG is claimed, persuasive medical evidence justifying

the medical necessity for the additional tests may be requested.

- 95805 MSLT- includes all the naps done in a single day. Only 1 unit of service should be submitted.

Coding Guidance

1. WatchPat must be billed as CPT code 95800 or 95801.
2. If a sleep study is performed for less than 6 hours, it should be billed with modifier 52. Providers should report the service provided with the appropriate reduced original charge.

Coding Information

CPT/HCPCS Codes

Group 1 (13 Codes)

Group 1 Paragraph

NA

Group 1 Codes

Code	Description
95782	POLYSOMNOGRAPHY; YOUNGER THAN 6 YEARS, SLEEP STAGING WITH 4 OR MORE ADDITIONAL PARAMETERS OF SLEEP, ATTENDED BY A TECHNOLOGIST
95783	POLYSOMNOGRAPHY; YOUNGER THAN 6 YEARS, SLEEP STAGING WITH 4 OR MORE ADDITIONAL PARAMETERS OF SLEEP, WITH INITIATION OF CONTINUOUS

Code	Description
	POSITIVE AIRWAY PRESSURE THERAPY OR BI-LEVEL VENTILATION, ATTENDED BY A TECHNOLOGIST
95800	SLEEP STUDY, UNATTENDED, SIMULTANEOUS RECORDING; HEART RATE, OXYGEN SATURATION, RESPIRATORY ANALYSIS (EG, BY AIRFLOW OR PERIPHERAL ARTERIAL TONE), AND SLEEP TIME
95801	SLEEP STUDY, UNATTENDED, SIMULTANEOUS RECORDING; MINIMUM OF HEART RATE, OXYGEN SATURATION, AND RESPIRATORY ANALYSIS (EG, BY AIRFLOW OR PERIPHERAL ARTERIAL TONE)
95805	MULTIPLE SLEEP LATENCY OR MAINTENANCE OF WAKEFULNESS TESTING, RECORDING, ANALYSIS AND INTERPRETATION OF PHYSIOLOGICAL MEASUREMENTS OF SLEEP DURING MULTIPLE TRIALS TO ASSESS SLEEPINESS
95806	SLEEP STUDY, UNATTENDED, SIMULTANEOUS RECORDING OF, HEART RATE, OXYGEN SATURATION, RESPIRATORY AIRFLOW, AND RESPIRATORY EFFORT (EG, THORACOABDOMINAL MOVEMENT)
95807	SLEEP STUDY, SIMULTANEOUS RECORDING OF VENTILATION, RESPIRATORY EFFORT, ECG OR HEART RATE, AND OXYGEN SATURATION, ATTENDED BY A TECHNOLOGIST

Code	Description
95808	POLYSOMNOGRAPHY; ANY AGE, SLEEP STAGING WITH 1-3 ADDITIONAL PARAMETERS OF SLEEP, ATTENDED BY A TECHNOLOGIST
95810	POLYSOMNOGRAPHY; AGE 6 YEARS OR OLDER, SLEEP STAGING WITH 4 OR MORE ADDITIONAL PARAMETERS OF SLEEP, ATTENDED BY A TECHNOLOGIST
95811	POLYSOMNOGRAPHY; AGE 6 YEARS OR OLDER, SLEEP STAGING WITH 4 OR MORE ADDITIONAL PARAMETERS OF SLEEP, WITH INITIATION OF CONTINUOUS POSITIVE AIRWAY PRESSURE THERAPY OR BILEVEL VENTILATION, ATTENDED BY A TECHNOLOGIST
G0398	HOME SLEEP STUDY TEST (HST) WITH TYPE II PORTABLE MONITOR, UNATTENDED; MINIMUM OF 7 CHANNELS: EEG, EOG, EMG, ECG/HEART RATE, AIRFLOW, RESPIRATORY EFFORT AND OXYGEN SATURATION
G0399	HOME SLEEP TEST (HST) WITH TYPE III PORTABLE MONITOR, UNATTENDED; MINIMUM OF 4 CHANNELS: 2 RESPIRATORY MOVEMENT/AIRFLOW, 1 ECG/HEART RATE AND 1 OXYGEN SATURATION
G0400	HOME SLEEP TEST (HST) WITH TYPE IV PORTABLE MONITOR, UNATTENDED; MINIMUM OF 3 CHANNELS

Group 2 (1 Code)

Group 2 Paragraph

Not Covered:

Group 2 Codes

Code	Description
95803	ACTIGRAPHY TESTING, RECORDING, ANALYSIS, INTERPRETATION, AND REPORT (MINIMUM OF 72 HOURS TO 14 CONSECUTIVE DAYS OF RECORDING)

CPT/HCPCS Modifiers**Group 1** (1 Code)**Group 1 Paragraph**

N/A

Group 1 Codes

Code	Description
52	REDUCED SERVICES: UNDER CERTAIN CIRCUMSTANCES A SERVICE OR PROCEDURE IS PARTIALLY REDUCED OR ELIMINATED AT THE PHYSICIAN'S DISCRETION. UNDER THESE CIRCUMSTANCES THE SERVICE PROVIDED CAN BE IDENTIFIED BY ITS USUAL PROCEDURE NUMBER AND THE ADDITION OF THE MODIFIER -52, SIGNIFYING THAT THE SERVICE IS REDUCED. THIS PROVIDES A MEANS OF REPORTING REDUCED SERVICES WITHOUT DISTURBING THE

Code	Description
	IDENTIFICATION OF THE BASIC SERVICE. MODIFIER CODE 09952 MAY BE USED AS AN ALTERNATIVE TO MODIFIER -52. NOTE: FOR HOSPITAL OUTPATIENT REPORTING OF A PREVIOUSLY SCHEDULED PROCEDURE/SERVICE THAT IS PARTIALLY REDUCED OR CANCELLED AS A RESULT OF EXTENUATING CIRCUMSTANCES OR THOSE THAT THREATEN THE WELL-BEING OF THE PATIENT PRIOR TO OR AFTER ADMINISTRATION OF ANESTHESIA, SEE MODIFIERS -73 AND -74 (SEE MODIFIERS APPROVED FOR ASC HOSPITAL OUTPATIENT USE).

ICD-10-CM Codes that Support Medical Necessity

Group 1 (17 Codes)

Group 1 Paragraph

Note: Diagnosis codes must be coded to the highest level of specificity.

95805 Covered for:

Group 1 Codes

Code	Description
G47.10	Hypersomnia, unspecified
G47.13	Recurrent hypersomnia

Code	Description
G47.14	Hypersomnia due to medical condition
G47.19	Other hypersomnia
G47.30	Sleep apnea, unspecified
G47.31*	Primary central sleep apnea
G47.32*	High altitude periodic breathing
G47.33*	Obstructive sleep apnea (adult) (pediatric)
G47.34*	Idiopathic sleep related nonobstructive alveolar hypoventilation
G47.35*	Congenital central alveolar hypoventilation syndrome
G47.36*	Sleep related hypoventilation in conditions classified elsewhere
G47.37*	Central sleep apnea in conditions classified elsewhere
G47.39*	Other sleep apnea

Code	Description
G47.411	Narcolepsy with cataplexy
G47.419	Narcolepsy without cataplexy
G47.421	Narcolepsy in conditions classified elsewhere with cataplexy
G47.429	Narcolepsy in conditions classified elsewhere without cataplexy

Group 1 Medical Necessity ICD-10-CM Codes Asterisk Explanation

*The MSLT should be performed after an attended PSG

* For patients with OSA treated with CPAP therapy, the clinician should ensure efficacy and adherence based on a review of downloaded data. If the patient is using non-CPAP therapy for OSA, then self-report of adequate use and efficacy of therapy should be confirmed before the MSLT.

Group 2 (26 Codes)

Group 2 Paragraph

95782, 95807, 95808 and 95810 Covered for:

Group 2 Codes

Code	Description
F51.3	Sleepwalking [somnambulism]
F51.4	Sleep terrors [night terrors]
G47.10	Hypersomnia, unspecified
G47.11	Idiopathic hypersomnia with long sleep time
G47.12	Idiopathic hypersomnia without long sleep time
G47.13	Recurrent hypersomnia
G47.14	Hypersomnia due to medical condition
G47.19	Other hypersomnia
G47.30	Sleep apnea, unspecified
G47.31	Primary central sleep apnea
G47.33	Obstructive sleep apnea (adult) (pediatric)

Code	Description
G47.34	Idiopathic sleep related nonobstructive alveolar hypoventilation
G47.35	Congenital central alveolar hypoventilation syndrome
G47.36	Sleep related hypoventilation in conditions classified elsewhere
G47.37	Central sleep apnea in conditions classified elsewhere
G47.411	Narcolepsy with cataplexy
G47.419	Narcolepsy without cataplexy
G47.421	Narcolepsy in conditions classified elsewhere with cataplexy
G47.429	Narcolepsy in conditions classified elsewhere without cataplexy
G47.50	Parasomnia, unspecified
G47.51	Confusional arousals
G47.52	REM sleep behavior disorder

Code	Description
G47.53	Recurrent isolated sleep paralysis
G47.54	Parasomnia in conditions classified elsewhere
G47.61	Periodic limb movement disorder
G47.9	Sleep disorder, unspecified

Group 3 (3 Codes)

Group 3 Paragraph

Medicare is establishing the following **limited** coverage for CPT/HCPCS code **95783, 95811 covered for:**

Group 3 Codes

Code	Description
G47.30	Sleep apnea, unspecified
G47.31	Primary central sleep apnea
G47.33	Obstructive sleep apnea (adult) (pediatric)

Group 4 (6 Codes)

Group 4 Paragraph

CPT codes **95800, 95801 and 95806** will be allowed when performed unattended in or out of a **facility** for the indications listed below:

CPT codes **G0398, G0399, or G0400** will be allowed when performed in the **home** for the indications listed below:

Group 4 Codes

Code	Description
G47.10	Hypersomnia, unspecified
G47.13	Recurrent hypersomnia
G47.14	Hypersomnia due to medical condition
G47.19	Other hypersomnia
G47.30	Sleep apnea, unspecified
G47.33	Obstructive sleep apnea (adult) (pediatric)

ICD-10-CM Codes that DO NOT Support Medical Necessity

N/A

ICD-10-PCS Codes

N/A

Additional ICD-10 Information

N/A

Bill Type Codes

Contractors may specify Bill Types to help providers identify those Bill Types typically used to report this service. Absence of a Bill Type does not guarantee that the article does not apply to that Bill Type. Complete absence of all Bill Types indicates that coverage is not influenced by Bill Type and the article should be assumed to apply equally to all claims.

N/A

Revenue Codes

Contractors may specify Revenue Codes to help providers identify those Revenue Codes typically used to report this service. In most instances Revenue Codes are purely advisory. Unless specified

in the article, services reported under other Revenue Codes are equally subject to this coverage determination. Complete absence of all Revenue Codes indicates that coverage is not influenced by Revenue Code and the article should be assumed to apply equally to all Revenue Codes.

N/A

Other Coding Information

N/A

Revision History Information

Revision History Date	Revision History Number	Revision History Explanation
06/26/2025	R6	Posted 06/26/2025 Under ICD-10 Codes that Support Medical Necessity Group 1 Codes added G47.31, G47.32, G47.33, G47.34, G47.35, G47.36, G47.37, and G47.39. Added Group 1: Medical Necessity ICD-10-CM Codes Asterisk Explanation: *The MSLT should be performed after an attended PSG and * For patients with OSA treated with CPAP therapy, the clinician should ensure efficacy and adherence based on a review of downloaded data. If the patient is using non-CPAP therapy for OSA, then self-report of adequate use and efficacy of therapy should be confirmed before the MSLT. Typographical and

Revision History Date	Revision History Number	Revision History Explanation
		grammatical changes made to AMA formatting. Review completed 05/15/2025.
07/27/2023	R5	Posted 07/27/2023-Review completed 06/20/2023 with no changes in coverage.
09/30/2021	R4	9/30/2021 Under Coding Guidance added information for billing with the 52 modifier. Under CPT/HCPCS Modifiers added 52 modifier with description. Under Updated Group 4 Paragraph: to state CPT codes 95800, 95801 and 95806 will be allowed when performed unattended in or out of a facility for the indications listed below:
08/26/2021	R3	08/26/2021: Under Coverage Guidance added WatchPat must be billed as CPT code 95800 or 95801. Updated Group 4 Paragraph: to state CPT codes 95800, 95801 and 95806 will be allowed when performed unattended in a facility for the indications listed below. Review completed 07/30/2021.

Revision History Date	Revision History Number	Revision History Explanation
07/29/2021	R2	07/29/2021. Review completed 06/22/2021.
11/01/2019	R1	Content has been moved to the new template.

Associated Documents

Related Local Coverage Documents

Articles

[A54953 - Independent Diagnostic Testing Facilities- physician supervision and technician requirements](#)
[↗](#)

LCDs

[L36839 - Polysomnography and Other Sleep Studies](#) [↗](#)

Related National Coverage Documents

NCDs

N/A

Statutory Requirements URLs

N/A

Rules and Regulations URLs

N/A

CMS Manual Explanations URLs

N/A

Other URLs

N/A

Public Versions

Updated On	Effective Dates	Status	
06/18/2025	06/26/2025 - N/A	Currently in Effect	You are here

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Keywords

- CPAP
- Sleep Apnea
- Narcolepsy
- Parasomnia